7 Human Rights-Based Psychosocial Support

7.1 Introduction

The conclusion from the previous chapter was that people with psychosocial problems should decide for themselves whether they consider themselves to be ill or not. An exclusively external assessment, the validity of which cannot be guaranteed, should therefore no longer be permitted. This step would remove a key building block for the legitimisation of coercive psychiatric measures, as these are often based solely on the opinion of a specialist with regard to the definition of illness.

Psychosocial support in accordance with individual preferences prevents restrictions and thus the violation of personal rights. In the following, however, the focus will be not only on prevention but also on the question of how human rights-based support can be imagined in a positive sense. What distinguishes human rights-based support from current and previous forms of care? What elements of support can be considered and in what form? The Quality Rights initiative of the World Health Organisation (WHO) has already proposed several aspects in this direction: respect for legal capacity, care without coercion, user participation, social inclusion, and recovery [1]. This direction will be developed more systematically in the following sections.

7.2 Human Rights Development in Mental Health Care

The human rights orientation and associated psychosocial support lie in meeting the preferences of service users and other affected persons. In a nutshell, it is about preference-based psychosocial support. Clinicians in the mental health care context should primarily follow what service users want. An essential aspect of the following remarks is the outline of ideal human rights-based support for the future, which, however, is already visible in parts today and has already been implemented to some extent.

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D. Richter, *Human Rights in Psychiatry*,
https://doi.org/10.1007/978-3-031-98635-2_7

Where the future is concerned, however, the present and the past must also be considered, as described in Chap. 3. In this context, I propose dividing mental health care since its beginning in the nineteenth century into three major phases in which the rights of the people affected by care were dealt with differently. The first phase, which lasted in Europe until around the 1960s/1970s, largely disregarded human rights. The human rights and civil liberties of people committed to asylums or institutions were generally ignored. The people concerned were mostly there against their will, and many had to spend years and sometimes decades in the institution. The medical procedure was generally determined by medical specialists who were able to rely on their professional and, where available, academic authority.

This was followed in the 1980s by a phase that is still dominant today. This phase can be characterised by the fact that human rights are acknowledged but can be restricted for medical reasons. Person-centredness and evidence-based approaches play decisive roles in this phase. The approach was to organise primarily medical psychiatry in the assumed interests of the users on the basis of scientific evidence of the forms of therapy.

Certain changes in terms of human rights are now to be seen, at least in outline. The next phase of psychosocial support with a distinct social psychiatry flavour should be human rights-based support. Its aim is to fully safeguard the fundamental rights and freedoms of the people receiving psychosocial support. Instead of "person-centred", the term "person driven" is now used [2].

7.3 What Support Preferences Do People with Psychosocial Problems Have?

During the human rights-ignoring phase of mental health care described above, there can be little talk of the preferences of the people affected. This should begin to change with the phase in which human rights were acknowledged but could nevertheless be considerably restricted for medical reasons—or more precisely, can still be restricted. There is now extensive research on treatment and support preferences in medicine in general [3, 4]. Similar studies are also available for psychiatry [5]. These findings reveal the following broader picture. First, pharmacological interventions are less popular with users than counselling or psychotherapy [6, 7]. Second, institution-based services are less favoured than outpatient or outreach services in their own home [8, 9].

As is usually the case with such survey studies that report group-related results, it must be taken into account that preferences are very heterogeneous. Not all people with psychosocial problems have the same ideas about how they want to be supported and what is important to them. The specific goals that people have presumably depend on numerous factors, such as whether and how ill they experience themselves or what goals they generally pursue in their lives. An interesting study from the United States identified three major groups of people with psychosis in this regard: people who want to achieve something in their lives, people who tend to

seek stability and people who are health-oriented [10]. The specific goals to be achieved also vary depending on the basic orientation. For example, a full-time job is expected to be sought predominantly by people who are more achievement-oriented. This group also tends to want to live in their own home and earn as much money as possible. What they have in common with the health-oriented group is that they do not want to experience voices or other unusual things. In contrast, this was less important for the stability-oriented group. This group also had less clear preferences for a high income, possibly because they had less pleasant experiences in the labour market.

These findings suggest that the life goals and support preferences of people with psychosocial problems are highly individualised and should be treated as such. This means that clinicians and other professionals should not assume conformist ideas ("people with X certainly want Y") but should instead—as with all people—enquire in each individual case and support the decision in question accordingly. This is particularly emphasised here because the ideas about appropriate goals and approaches very often differ among professionals, users and other stakeholders, such as relatives [11].

Furthermore, it is important for me to emphasise that the characterisation of today's mental health care as a purely neoliberal commercial venture for the pharmaceutical industry and the "psy-professions" (e.g. [12, 13]) misses the core of the problem. In the course of the book, I described the social constructivist background to "illness" and cited the sometimes-low effectiveness of therapies. Moreover, it is of course true that entire industries and professions live from medicalisation and psychologisation and therefore benefit from it. However, the call for a de-medicalisation of psychosocial problems and the characterisation of, for example, antidepressants as a myth [14] leads to an epistemic injustice towards those who, with the help of professional support and pharmacology, can lead to a stable and liveable life. If it helps, benefits and does not harm these people from their perspective, what should speak against it?

Medicalisation and psychologisation contribute significantly to coping with the sometimes difficult and aversive conditions of life in modern society. It is certainly not wrong to emphasise "... that the concept of an illness is not required to understand the aetiology of a large number of problems currently considered to represent some form of illness..." ([15]: 11). From this statement, the authors conclude that psychopathology should be depathologised. However, the same applies here as before: If people with psychosocial problems interpret them as "illness", as happens millions of times, and this helps them to understand their problems and to deal with them, what should speak against it? A medicalised and psychologised perspective on personal problems is the dominant view in the Global North today. A medicalised and psychologised perspective therefore has the same right to exist as the rejection of this perspective. From a social psychiatry and mental health nursing perspective in particular, it should be possible for the perspectives to exist and stand side by side on an equal footing. If this is not possible, the epistemic injustice that we criticise from an exclusively biomedical perspective would be promoted.

7.4 From Shared Decision-Making to Supported Decision-Making

On several occasions, the United Nations Convention on the Rights of Persons with Disabilities (UN CRPD) has created new foundations for the understanding of disabilities and impairments as well as for psychiatric care. This applies in particular in the context of decision-making by professionals and affected persons or users. The background here is the legal capacity of people with disabilities, which can generally be restricted by national or regional legal systems, often in a significant way. In contrast, Article 12 of the UN CRPD emphasises the goals "...that persons with disabilities have the right to recognition everywhere as persons before the law" and that measures should be taken "... to provide access by persons with disabilities to the support they may require in exercising their legal capacity" [16]. In other words, people with disabilities should be able to make their own decisions about their legal affairs, and professionals should support them so that they can actually do so. This is essentially the legal view, which could lead to the abolition of legal representatives such as guardians or legal carers [17] if they are appointed against the will of the person concerned.

However, this principle can, of course, be applied not only in connection with legally relevant decisions. As in the field of physical medicine, where this approach is sometimes called "informed decision-making" [18], this approach can and should also be the norm in psychosocial care—up to and including decisions on measures that could involve coercion [19]. From a human rights perspective, which favours decisions on the basis of will and preference, it can generally be assumed that people with psychosocial problems can decide for themselves what is the right choice for them in terms of support, care and treatment, even if they need assistance to come their right choice. This is what 'supported decision-making' means.

In some countries, physical care has already gone further than psychiatric care. The British *Royal College of Surgeons* (RCS) issued standards back in 2016 that declare supported decision-making to be a principle [20]. The background to this was an obstetric case in which a patient was not informed of certain risks and her child was born with birth defects [21]. The UK Supreme Court subsequently ruled that it was not permissible to provide information based on clinical judgement but that all such information should be disclosed for the purposes of informed choice. Interestingly, the standards required by the RCS also include the option of no treatment. According to the document, if patients are capable of judgement, it must be accepted that they do not receive treatment, even if this is contrary to clinical judgement and medical ethics.

In the relevant psychiatric research literature, institutional or communicative approaches such as "Open Dialogue", peer support or Soteria houses and wards are among the implementations of supported decision-making [22]. This is hardly surprising given that the same approaches are regarded by the World Health Organisation (WHO) as exemplary for the implementation of a human rights-based approach in line with the UN CRPD [1]. As will become clear in a moment, these approaches are not considered particularly effective in terms of the evidence-based psychiatry

practised today. Apart from the fact that this may be due to methodological circumstances, the question remains as to whether scientific evidence should always take precedence over the preference of the user in psychosocial support [23]. In supported decision-making, there is, as is easy to imagine, a certain risk from the clinician's point of view that the beneficiaries will decide against the scientific evidence.

In this context, however, it is important to remember various aspects that are relevant here. First, it can be assumed that many users are strongly oriented towards biomedicine and are prepared to accept scientific evidence as decisive in terms of decision-making [24]. Second, however, it should be remembered that the evidence for the effectiveness of psychiatric treatment is not particularly strong, as I show in Chap. 4. In this respect, the argument that one must follow evidence-based psychiatry is not particularly convincing. Third, it should be remembered that the methods of evidence-based medicine (EBM) today have moved quite far away from how they were developed in the 1990s. Even back then, the danger was recognised that EBM would be applied in a cookbook-like manner, with decisions being made solely on the basis of studies [25]. However, EBM, in the sense of its original approach, was based on a high value of clinical expertise, which should be incorporated into the medical decision-making process. Furthermore, the entire decision-making process should be characterised by "... the more thoughtful identification and compassionate use of individual patients' predicaments, rights, and preferences in making clinical decisions about their care" ([26]: 71). Supported decision-making is obviously not thus far removed from EBM in the initial sense.

7.5 Supported Decision-Making: Preference over Evidence

Evidence-based support or preference-based support is therefore one of the crucial questions in connection with the decision-making of people with psychosocial problems. As can be expected, my answer is that preferences are the most important factor, and scientific evidence comes into play when service users have a support or treatment preference that has been scientifically verified. What does that mean in concrete terms? The principles of Soteria treatment, as implemented in the 1970s and 1980s in the United States and then later in Europe, included acute psychiatric care without neuroleptics or later with a relatively low antipsychotic pharmacological dose [27]. However, as is easy to understand, this low-dose therapy is less successful in achieving the treatment goal of preventing a first or second psychotic relapse [28].

Now the conventional conclusion is as follows: especially at the beginning of psychotic development, treatment should not be carried out at a low dosage. On the other hand, from my perspective, the question would be how relevant the risk of a psychotic relapse is from the perspective of the person affected, especially against the background of the not inconsiderable side effects of a higher dose. If relapse prevention is not a high priority, this should also be reflected in the dose of antipsychotics—if medication is accepted at all. However, if relapse prevention is the top priority, the person should be informed about the effects and side effects of the dose

in an evidence-based manner to make the decision criteria more transparent and facilitate the decision. This is where preferences come into play again. A higher antipsychotic dose is associated with more side effects [29]. If this is accepted in view of the prioritisation of relapse prevention, the evidence-based form of therapy and dose of medication would ultimately come into play here. If the side effects are not acceptable, alternative non-drug support strategies should be discussed.

It is perfectly possible to formalise the procedure outlined above via a generic algorithm. Figure 7.1 describes the decision paths. From the point of view of the person concerned, there is a psychosocial problem (e.g. psychotic experience) for which a preferred solution is sought (in this example, the solution could be called Open Dialogue procedure or medication). As the name suggests, the Open Dialogue process is an outreach, systemic and communication-based process that attempts to reach decisions in the event of a severe mental health crisis with the involvement of all parties [30]. Therefore, evidence may be available for the preferred solution (medication) or not (Open Dialogue). If the person opts for the solution without evidence, this is nevertheless the procedure, and Open Dialogue is used. If they decide in favour of evidence, all information regarding effects and side effects should be disclosed. The next step is to assess whether the evidence is acceptable,

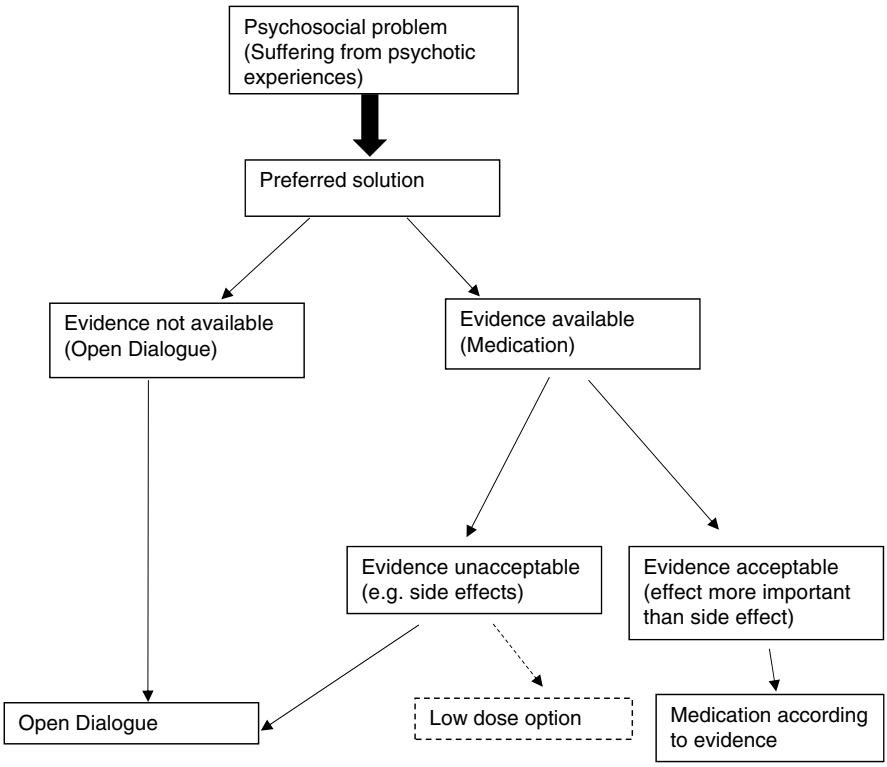

Fig. 7.1 Preference versus evidence—decision algorithm

i.e. whether the effect of the drug is considered sufficient, even in view of the side effects. If this is not the case, non-drug solutions should be considered, or a lower dose should be prescribed. Otherwise, the medication is administered in accordance with the evidence.

In other words, evidence-based decisions are always relevant in the care of people with psychosocial problems if the goals favoured by the users have been evaluated with regard to their achievement via appropriate research methodology. This approach is a reversal of the usual approach in many parts of mental health care, but it is compatible with the initial evidence-based medicine of the 1990s.

According to current research, a good therapeutic alliance between a professional and a person with a psychosocial problem is crucial for the effectiveness and sustainable implementation of supported decision-making [31, 32]. Trust is therefore key. Affected persons must be able to trust that the professional will provide all relevant information and communicate this in an appropriate and, above all, comprehensible form. It must also be possible to trust that the preferences will ultimately be implemented in the measures that are deemed necessary.

A fundamental problem in the therapeutic alliance between clinicians and people with psychosocial problems, which can only be overcome with trust, is the undoubted power asymmetry that exists. Professionals usually have more and better information at their disposal, and they generally also have the ability to achieve their goals through communication. This may be the reason why peer counselling is generally held in high regard by those affected or those who use it. In contrast to peers, professionals are perceived as more goal-oriented, whereas peers are seen as more process-oriented [33]. Presumably, the preferences of users can be discussed and weighed more openly and appropriately in discussions with peers than is the case with conventionally trained staff.

7.6 Human Rights-Based Psychosocial Support: Person-Centred and Preference-Based

The history of human rights in psychiatry can be divided into three phases, as described above: disregarding human rights (phase 1), acknowledging human rights (phase 2) and human rights as the basis of support (phase 3). The right-hand column of Table 7.1 lists what I consider to be the essential aspects of human rights-respecting psychosocial support. These aspects are supplemented by what I consider to be essential in the earlier phases 1 and 2. In such a concise list, nuances and certain variations are naturally lost, especially as the phase of disregard for human rights spans approximately two centuries.

To respect human rights, the *focus of support* must no longer be on institutions, as in the past (phase 1), or on person-centredness, as at present (phase 2), but on person-driven management (phase 3). As correct and important as person-centredness was, its shortcomings became clear, for example, in that psychiatric coercion could certainly be justified as a person-centred measure [34].

Table 7.1 Human rights phases in psychiatry

	Disregarding human rights (Phase 1)	Acknowledging human rights (Phase 2)	Human rights as a basis (Phase 3)
Focus	Institution-centred	Person-centred	Person-driven
Decision criterion	Eminence (medical-scientific authority)	Scientific evidence	Preference of the person
Ethical approach	Social norms, interest of the family	Well-being of the person concerned	Will and preference of the person concerned
Legal basis	Mainly involuntary	Involuntary hospitalisation and treatment in certain circumstances; formal voluntariness, but widespread informal and administrative coercion	Exclusively voluntary
Legal capacity	Mostly refused	Restricted under certain circumstances	Extensively granted
Coercion	Predominantly forced placement and treatment	Forced placement and other measures not the rule, but happen often	No coercion in care; normality principle; quarantine in case of ongoing risk to others
Place of care/ support	Hospital/institution	Hospital and community	According to preference
Supply/support model	Institutional psychiatry	Social and community psychiatry	Psychosocial support according to preference (also exclusively medical treatment)
Disease model	Psychodynamic, biomedical	Biopsychosocial	Psychosocial problems—plurality of models
Health model	None	Absence of symptoms, later quality of life	Psychosocial health—individual priorities, including recovery
Importance of symptoms	Suppressed	Treated	Relevance and handling depending on preference
Importance of the diagnosis	Broad diagnostic groups, prognostic function	Central feature of the treatment and the organisation of the treatment	According to preference and disease model

Professional approach	Accommodation	Treatment/care	Support according to preference (treatment/care/coaching/peer support)
Objective	Monitoring the person	Symptom management	Individual goals (according to preference)
Main intervention	Compulsion/physical therapies/medication	Medication/psychotherapy for special indications	According to preference
Medication	Often by force/without consent	Prescription, possibly against the will	Professionally supported self-determined intake/non-intake
Professions in the support system	Dominance of the medical profession	Multi-professional team under medical management	Multi-professional team without professional group dominance; inclusion of service users as peer support; ideally inclusion of peers in the management of programmes
Needs/requirements	Physical requirements, if any	"Objective" psychosocial needs/requirements (plus satisfaction and quality of life)	Subjective psychosocial and/or physical needs
Deficits/strengths	Deficit-orientation	Balance of deficits and resources	Predominantly resource-orientated
Risk orientation	Harm/risk avoidance	Risk minimisation/risk management	Dealing constructively with risks (positive risk-taking)
Balance of power	In favour of the professionals	Intended but not realised balance	According to preference
Social goal	Social exclusion	Social integration	Social inclusion
Rehabilitation	None (living and working in the institution)	Stepladder/linear continuum	Supported inclusion
Research	Users as involuntary objects of research	Mental disorder including the user's perspective (e.g. satisfaction and quality of life)	Active involvement of users in research planning and implementation
Professional training/university teaching	Service users as exposed objects	Service users as objects of clinical training	Service users as teachers

Person-centredness does not exclude coercion. With person-driven control, this is the sole responsibility of the person affected by any action.

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This means *that the decision criterion* for what should happen in the support is the preference of the person concerned. In earlier times, this was the eminence in the person of the medical or scientific authority [35] and, later, the scientific evidence. In essence, there must be a change from evidence to preference.

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The importance of the preference of the person concerned is also prominent in the central *ethical approach* of human rights-based phase 3. Whereas in phase 1, it was the social norms and the interests of the family, this changed in phase 2 to the supposed benefit of the person concerned, whereas now it is the will and preference of the person, as is also expressed in the UN CRPD.

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The *legal basis* for support, care and treatment must therefore be exclusively voluntary. It must no longer be determined by formal, informal or administrative coercion. In the institution-centred phase, which disregards human rights, people were predominantly treated involuntarily, whereas in the present day, this is predominantly done formally on a voluntary basis. Nevertheless, many people are affected by measures taken against their will, not to mention widespread informal and administrative coercion.

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This means that the *legal capacity* of the person concerned should no longer be restricted in the future, as is still the case today in almost all legal systems around the world. Legal capacity must be fully granted, as required by Article 12 of the UN CRPD [36].

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Coercion was widely used during the human rights abuse phase. This is less the case today. The use of coercion, in any form, is fundamentally inadmissible in support of human rights. Exceptions to this are self-binding directives in which people have determined what should happen if, in their judgement, they can no longer make appropriate decisions [37]. Such agreements are also called "Ulysses clauses". As a reminder, in Homer's "Odyssey", the protagonist had himself tied to the ship because he feared that he would not be able to resist the sirens' enticing song. The next chapter explains what principles and options a modern society must have to be able to deal appropriately with people who pose risks to themselves or to others. It also describes the dilemma situations that can arise.

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In the past, *the place of care and support* was more or less exclusively the asylum/hospital or the institution, as it was called at the time. In the context of deinstitutionalisation and dehospitalisation during psychiatric reforms, services

increasingly shifted to the community, and pure institutional psychiatry became the community psychiatry that is predominantly found today [38]. However, hospitals still have a central function in many regions, as some services are only available there. Human rights-based support would stipulate that the place of care is up to the service user's preference.

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In the context of support that respects human rights, the *type of support* is up to the preferences of the person concerned, too. This does not preclude people from seeking conventional pharmacological and/or psychotherapeutic treatment, nor does it preclude hospital treatment; the decisive factor is the person's preference.

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In terms of *disease models*, there has been development from the biomedical and psychoanalytical concepts of earlier decades towards a—at least envisaged—bio-psychosocial model [39]. However, the latter has been controversial for some time [40], and the question is to what extent it has been applied at all [41]. Preference-oriented support relies on diverse models of psychosocial problems, especially as there is no obvious argument in favour of one particular model over others [42]. According to the spectrum model of psychosocial problems described in the previous chapter, preference is also decisive here.

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This also corresponds to the concept of psychosocial *health*. This concept first developed in the person-centred phase, which took human rights into account, in the direction of the absence of symptoms, as typically recorded in the context of standardised psychopathological assessments. Only around the turn of the last century were further aspects such as quality of life surveyed [43]. Psychosocial health in the context of support that respects human rights is not only an individual concept that can also include the absence of symptoms but also aspects such as personal recovery, which is about finding meaning despite or perhaps even because of psychosocial problems [44].

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The concepts of illness and health also correspond to the different ways of dealing with the phenomena called *symptoms* in the respective phases. Whereas in the era of disregard for human rights, the primary aim was to suppress the phenomena without treatments being available, in recent decades, they have become the actual goal of treatment. The predominant pharmacological approaches are aimed at assessing psychopathological symptoms. The relevance of these phenomena, whether they are even assessed as "symptoms" of an "illness", is, in turn, a matter for the affected persons themselves in a support system that respects human rights. The frequently cited example of hearing voices and dealing with them can show the path that needs to be taken.

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The different approaches to *diagnoses* also correspond to the concepts of illness and health. In the early days of classification systems, these systems were rather broadly based and had primarily prognostic functions [45]. The diagnosis subsequently became the central feature of treatment, organisation and reimbursement. Special wards, in which people with the same diagnoses are treated, have been increasingly implemented in recent decades. However, many of these wards have already been abandoned in favour of the so-called track system [46]. The track system claims to offer more flexibility and autonomy. In a support system that is person-centred and preference-based, this should be determined entirely by the people concerned. Diagnoses lose their significance if the individual disease model does not go hand in hand with them. However, diagnoses can be very valuable in providing affected people with a better understanding of their psychosocial problems. This means that for many users, a diagnosis can be meaningful and useful; for others, it can mean stigma and discrimination [47].

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Now to the support itself: The general *professional approach in* the early phase of psychiatry consisted primarily of the accommodation and segregation of the affected person from their social environment, and above all, in separation from the family ([48]: 89ff.). For a long time, there were no forms of therapy available that were even minimally effective. This changed only in the second half of the twentieth century, where treatment and (nursing) care were the central professional approaches in the phase that took human rights into account. However, this also implied treatment against the person's will. Therefore, support that respects human rights requires modified and additional support options that can be chosen according to preferences, such as coaching, peer support and communicative interventions such as Open Dialogue. In particular, the inclusion of peers as people also affected by psychosocial problems can be an essential step toward building trust in non-medicalised support systems, for example, in dealing with suicidal tendencies [49].

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The predominant *objective* in the human rights disregarding phase was monitoring. This was replaced by the treatment goal of symptom management. Psychopathology was the focus of treatment in the human rights-acknowledging phase. Individual goals, which can also lie beyond phenomena and symptoms, should be the focus of support in the future. These can be not only aspects of everyday professional or social life but also non-pharmacological forms of coping that make it easier to address certain phenomena [50].

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The main *interventions* in the early phase were often coercion as an end in itself to monitor and control people, followed by physical interventions such as lobotomy, a psychosurgical measure that is now regarded as one of the greatest mistakes in medicine [51]. The second phase, which took human rights into account, initially continued to work with biological interventions, even without the consent of the patients concerned. Furthermore, pharmacological therapy was the primary

measure, although individual indications were and are treated psychotherapeutically. However, to respect human rights by taking into account the individual preferences of the people receiving support, further options are required in the proposed next phase, which have already been mentioned above in connection with professional approaches.

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Relevant *forms of medication* were not available in psychiatry until the 1960s [52]. As described, these were often administered by formal and informal coercion without the actual consent of those affected. In the human rights acknowledging phase, this was duly prescribed, but forced medication has not disappeared to this day. In Switzerland, more than 3% of inpatients recently received forced medication [53]. It goes without saying that this should no longer happen if human rights are respected. Rather, the aim is for people receiving support to learn to use medication in a self-determined way. If medication is helpful, it can be an important component of psychosocial stabilisation. If they are experienced as unhelpful or even detrimental to health, they need to be handled differently, which may include professionally supervised discontinuation of medication [54].

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In the era of psychiatry, which disregarded human rights, the medical profession developed as the dominant *profession*. The second major professional group was initially the untrained "lunatic attendant", from which mental health nursing developed. In the current phase, we find the so-called multi-professional team, which—with certain exceptions—is still led by the medical profession for historical and legal reasons. To do justice to the preferences of the people to be supported, a multi-professional team without a dominant professional group but with the involvement of peers is required as a minimum. Ideally, peers will also be involved in the management of support services in a further step [55]. However, these roles also require a certain amount of experience.

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In the first centuries of psychiatric care, the focus was not on mental *needs and requirements* but, at best, on physical needs. Even these were often not dealt with in the best interests of the people being cared for, given the brutal coercive measures and later interventions and medication administered against their will. In connection with the concept of person-centredness, it became clear in the phase in which human rights were taken into account how much person-centredness was based on "objective" indicators of need and not on the subjective needs of the people concerned. This then changed to some extent with the consideration of quality of life and user satisfaction in the 1990s [56]. In human rights-based support, however, subjective psychological, social and physical needs should be at the centre of care. The dilemmas caused by this, for example, in social law matters, are discussed in the following chapter.

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The history of psychiatry is also a history of *deficit orientation*; even certain sexual orientations were characterised as mental deficits [57]. We are currently experiencing a certain tendency towards resource orientation in the phase in which human rights are taken into account [58]. Overall, however, we can only assume a balance between deficit orientation and resource orientation. This should continue to change towards a primary resource orientation if human rights are not only taken into account but also respected. Even if it is perhaps not politically opportune at present to refer to deficits and problem situations, this cannot be dispensed with. Without deficits and problems, no support would be utilised. The difference from current practices is the significance of resources and deficits, which should change in favour of resources.

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As the French sociologist Robert Castel noted, the history of psychiatry has seen a change in the way people categorised as mentally ill have dealt with the harms and *risks* they appeared to pose. This change took place from the perception of harm to risk [59]. As already indicated on various occasions, many people with psychosis were initially generally regarded as dangerous to the public. Later, in the human rights-acknowledging phase 2, risk assessment and risk management were and still are at the centre of attention; I described this in connection with the legitimisation of coercion in Chap. 2. In a human rights-based phase, this should have changed in the direction of a positive approach to risk (*positive risk taking*). In the future, it will not be a question of avoiding risks but rather of consciously taking risks under certain circumstances and learning from any problems and mistakes. At present, many employees in mental health services still find this quite difficult, as risk avoidance0 is still a widespread attitude [60, 61]. Certainly, this also has to do with possible legal consequences if damage occurs. The legal system is not geared towards a positive approach to risk.

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For many centuries, the relationship between clinicians and service users or people affected by psychiatry was an asymmetrical *balance of power* in favour of the staff. In fact, this is still the case today. Although concepts such as the joint/shared decision-making described above have aimed to achieve a balance of power [62], this approach has rarely been implemented in routine care [63]. However, shared decision-making does not go far enough in terms of the UN CRPD. This calls for supported decision-making [64], which changes the power asymmetry in favour of the user. It is also important to take preferences into account here. Not all users want to make self-determined decisions; some also agree with professionally determined decisions about their treatment and support.

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The social goal of psychiatry in the phase of human rights abuse was social exclusion. Not all but many people had to spend several years to decades in institutions, separated from their social environment against their will. In the phase that respected human rights, the goal of social integration developed [65]. The

human rights goal of social inclusion [66], which essentially means adapting the social environment to the needs of the person to be supported, is already recognizable to some extent in today's practice. Within Supported Employment programs, for example, we see such adaptations of the working environment to people's needs.

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The models of *psychiatric rehabilitation* correspond with social goals. Initially, virtually no rehabilitation was aimed at living and working in the hospital or institution, and occupational therapy was seen as something that was intended to support "healing" [67]. The rehabilitation goal of integration was to be achieved in the subsequent phase via the so-called step ladder or linear continuum principle [68]. As with medical rehabilitation in general, there was a conviction that increasing stress and strain would enable the person concerned to regain the necessary competences by adapting to the social environment. Today, we know that most users do not achieve their social integration goals by climbing the ladder. Furthermore, inclusion does not mean adapting to the environment but adapting the environment to the person's needs in the sense of supported inclusion [69]. Again, Supported Employment is a good example of this adaptation.

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Chapter 3 has already described how service users or people affected by psychiatry generally became involuntary objects of psychiatric *research*; this was common during the human rights abuse phase. The issue not only affected psychiatry but also disenfranchisement was greater here than in other areas of medicine, as most people affected by research were involuntarily accommodated and treated. This was not to formally change psychiatry until the 1964 "Declaration of Helsinki" by the World Medical Association [70]. Today, the requirement of informed consent for study participants applies to all medical research. In the human rights phase, increasing emphasis was also placed on the perspective of users, for example, through surveys on satisfaction and quality of life. Research that respects human rights also requires the active participation of users/affected persons in research projects, which are ideally also developed and managed by them [71].

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A similar story can be observed for the role of users/affected persons in *professional training and university teaching*. Here too, for decades, users/affected persons were also objects that were presented to students and probably also to the wider public in lecture theatres [72]. This practice changed over time, but I can still remember so-called tribunal ward rounds, in which service users sat opposite several clinicians seated in a semi-circle. Such ward rounds were used in the presence of the users for training purposes, among other goals. However, to make the perspectives of users/affected people clear in the context of human rights-based support, active and ideally leading involvement of affected people is needed, as in previous research [73]. The motto should also apply to training: "Nothing about us without us."

7.7 Conclusions: Building Blocks of Person-Centred Support

In my opinion, the contrast with previous and current practices in psychiatric care has made it possible to highlight the steps towards human rights that are already visible today but also those that need to be taken. In many places, the simple solution is that the preferences of the users and those affected should decide what to do and what not to do.

However, the implementation of such preference-based support faces many hurdles. Numerous legal, ethical and practical problems and dilemmas must be recognised and dealt with along the way. The penultimate chapter addresses the specific difficulties and possible solutions.

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